

"It is not our job to educate you": Exploring Perceptions of AI-Integrated Menstrual Tracking Apps Through Speculative Reflection

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Abstract

Medical misogyny contributes to dismissal and normalization of pain in menstrual healthcare, often leaving AFAB (assigned female at birth) people with undiagnosed conditions for months or years. Meanwhile, menstrual tracking apps (MTAs) are increasingly integrating artificial intelligence (AI) to identify patterns that may evidence menstrual conditions in clinical settings. However, limited research explores whether AI-integrated MTAs challenge or reinforce medical biases. In this study, we worked with seven AFAB people aged from 24 to 39 years old in a speculative reflective writing process to explore AI-integrated MTAs through a feminist HCI lens. Thematic analysis was used to analyze reflections, and four themes were identified 1) compulsory AI-generated evidence; 2) quantifying menstrual experiences; 3) reinforcing reproductive labor; 4) a sociotechnical hierarchy of trust. Our findings show that AI-integrated MTAs could marginalize users with menstrual conditions, and frameworks are needed to evaluate how apps might perpetuate or challenge biases in menstrual healthcare.

CCS Concepts

• **Human-centered computing** → **Human computer interaction (HCI)**; Ubiquitous and mobile computing; • **Applied computing** → *Health informatics*.

Keywords

Menstrual Conditions, Feminist HCI, AI, Critical Health Technology

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1 Background

Historically, assigned female at birth (AFAB) people, including women, transgender, and non-binary people, have experienced marginalization in medicine, with gendered conditions often delegitimized [13]. White male-dominated institutions continue to dismiss AFAB people's experiences, which has led to systemic failures in addressing health concerns [17]. In England, healthcare is provided by the National Health Service (NHS), a tax-funded system offering citizens free universal care [19]. The NHS provides primary care for AFAB people suspecting or with a menstrual condition like endometriosis, premenstrual dysphoric disorder, and polycystic ovary syndrome [20]. However, a 2024 parliamentary inquiry [33] revealed that accessing diagnosis and treatment can take years, leaving people to endure symptoms which interfere with their daily lives, while their conditions worsen. AFAB people also report disbelief and symptom normalization, as conditions like endometriosis and PMDD are often misdiagnosed [5]. This systemic neglect reflects medical misogyny and gaslighting, where symptoms are downplayed or dismissed [14, 28].

In parallel, menstrual tracking apps (MTAs) have proliferated, designed to support AFAB people in understanding their menstrual health, with 50 million worldwide users [31, 34]. However, research has revealed concerning biases embedded in MTAs. Fox and Epstein [6] found that MTAs assume gender identity and regular cycles, use gendered design aesthetics, and prioritize fertility tracking. The findings showed that participants primarily tracked menstruation over fertility, using non-digital methods. The normative cycle assumptions can lead to misinterpretation of atypical menstrual experiences as errors rather than valid experiences [8]. Users have raised concerns about data privacy practices in MTAs [23], particularly as many popular MTAs are founded by men [24], and 84% sell data to third parties [22].

Other research analyzing interaction design of MTAs has shown how apps might perpetuate marginalization. Tylstedt and colleagues [29] found that MTAs use calendars and timelines which reinforce



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tracking for reproductive control, encourage extensive data entry, and present algorithmic guesses as truths. These practices can lead to cycle assumptions and position AFAB people as responsible for tracking their reproductive health [15].

MTAs are integrating artificial intelligence (AI) to predict cycles and flag symptoms of potential health conditions [25], promising bodily awareness and clinical evidence generation [26, 30]. However, the integration of AI into MTAs may amplify existing concerns. Mirzaliyeva [18] found that AI-integrated MTAs provided inconsistent cycle predictions, particularly for people with irregular cycles. Participants also reported overwhelming symptom alerts, unhelpful chatbot interactions, and data privacy concerns. These findings suggest that AI algorithms may be designed around normative patterns, excluding those with menstrual conditions or concerns, whose cycles and symptoms may fall outside the norm [2].

While research has identified that MTAs could support people with menstrual conditions in recording cycle dates and symptoms [12], limited research has explored whether AI-integrated MTAs challenge or reinforce biases in the context of medical dismissal. In this paper, we used reflective writing design artifacts to explore AFAB people's perceptions of AI-integrated MTAs. We posed the following research question:

How do AFAB people reflect on the potential role of AI-integrated MTAs in navigating medical dismissal of menstrual health concerns?

We worked with seven AFAB people aged 24 to 39 years old. Participants did not disclose menstrual condition diagnosis, menstrual concerns, or experience of dismissal. They interacted speculatively with a series of postcards designed to prompt reflection on medical dismissal and AI-integrated MTAs. We used a thematic analysis [4] to analyse their responses.

The findings revealed: 1) AI-integrated MTAs could validate dismissed symptoms but there were concerns raised around AI-generated evidence becoming 'compulsory'; 2) AI might reduce embodied menstrual experiences to quantified data; 3) technologies could reinforce reproductive labor of tracking to people with menstrual conditions rather than reduce barriers to care; 4) AI-integrated MTAs might lead to hierarchies of trust across menstrual healthcare.

Our main contributions are: 1) demonstrating how AI-integrated MTAs may further marginalize people with menstrual conditions or concerns; 2) using slow methods to reflect on technologies which are being rapidly implemented; 3) identifying a need for design frameworks to evaluate how AI-integrated MTAs might perpetuate or challenge biases in menstrual healthcare before deployment.

2 Method

2.1 Positionality

We acknowledge that researcher demographics, values, perspectives, and experiences influence the interpretation of qualitative data [3]. The present research was carried out by a team of four researchers based in England.

The principal investigator is a White British, working-class, cis-gender, straight woman who grew up in a single-parent household in England. She has a menstrual condition and has lived experience of medical misogyny. Drawing on lived experience can allow for

more situated and meaningful inquiry [27], as such, her positionality informed her approach to design postcards and prompts that centred embodied pain expression, the labour of symptom tracking, and healthcare activism. This positionality also informed her approach to analyzing participants' responses to dismissal and AI technologies in menstrual healthcare.

2.2 Participants

An opportunity sample was used to recruit seven participants interested in exploring the use of AI-integrated MTAs to challenge medical dismissal. The eligibility criteria were to be over the age of 18 and assigned female at birth. Participants did not disclose menstrual condition diagnosis, menstrual concerns, or lived experience of dismissal. However, this does not indicate absence of experience. Participants were aged from 24 to 39 years old ($M = 30$). One participant did not disclose their gender identity, while six indicated they identified as a woman. One participant had not used an MTA before, while six indicated they had. MTA use was varied, with two participants using an app monthly, one weekly, and one had deleted the app. Participants were given pseudonyms to protect their anonymity.

2.3 Design Artifacts

To engage participants in speculative reflection on AI-integrated self-tracking in menstrual conditions, we developed a physical artifact pack titled 'Return to Sender' (see Figure 1). Each pack consisted of a kraft paper gift bag tied with a red ribbon containing an instruction card and four red envelopes numbered 1-4, each housing an illustrated postcard.



Figure 1: Return to Sender Pack Design. © Respective copyright holders via Canva

The postcards aimed to take participants on an chronological journey through four speculate reflective writing prompts, designed to guide participant reflection across a progression: from embodied experience to systemic critique. The prompts were centered on medical dismissal, AI-integrated MTA use, and systemic change (see Table 1). This approach allowed participants to draw on their knowledge and perspectives, without requiring disclosure of medical conditions or personal experience. Postcard 1 mirrored the cultural context of rushed clinical encounters and medical censorship that restricts embodied pain communication [7, 32]. Postcard 2 explored whether AI outputs validate or homogenize symptoms,

and how trust in algorithmic authority compares to clinical expertise given patterns of medical dismissal [1]. Postcard 3 examined the emotional and practical labor involved in menstrual symptom tracking [16, 21]. Postcard 4 encouraged healthcare activism by inviting participants to envision transformed systems of care. While postcards 1-3 used open-ended questions, postcard 4 takes an explicit theoretical stance, grounded in literature on medical dismissal in menstrual healthcare [14, 28, 33], encouraging participants to reflect on change needed to support people with menstrual conditions and concerns.

Table 1: Return to Sender Postcard Prompts.

Postcard	Prompt and Rationale
Postcard 1: "Dear Doctor, Can You Hear Me?"	"Write a message to a healthcare professional describing menstrual pain which may be attributed to a menstrual condition, without using the words 'pain' or 'period.'"
Postcard 2: "Do You Believe Me?"	"Imagine you have to log your symptoms in an AI period tracking app to inform your doctor of your symptoms. If the algorithm summarised your symptoms in one sentence, what would it say? Would you trust it more than a doctor?"
Postcard 3: "Logging and Reliving"	"Imagine you are writing a message to a health professional, a loved one, or someone else, describing what logging your pain is like. How do you feel about having to track your pain everyday to get support?"
Postcard 4: "Wish You Were Listening"	"Many women experiencing painful menstrual conditions find their symptoms 'normalised' and their 'pain dismissed'. This leads women to track symptoms using AI period tracking apps to validate their pain and receive diagnoses. Write a message to the healthcare system on what could change to support women."

2.4 Procedure

This study was approved by the host university's ethics committee. First, participants completed an online questionnaire providing informed consent and demographic information including age, gender identity, and MTA usage. Then, participants were each given a postcard pack to complete within two weeks. Data was collected between March 2025 to November 2025. After an initial collection period in spring 2025, recruitment paused and resumed in fall 2025. The postcard prompts and study protocol remained consistent throughout both phases. We used thematic analysis [4] to analyse AFAB people's reflections on how AI-integrated MTAs might reinforce or challenge medical dismissal. The coding took place over seven days, dedicating one day to each participant's submission to follow the narrative in their reflections. Following initial coding, the researcher returned to the full dataset to review themes across all submissions, ensuring consistency of interpretation.

3 Findings

Here, we present four themes in relation to AFAB people's reflections on AI-integrated MTAs.

3.1 Compulsory AI-Generated Evidence

Participants recognized AI-integrated MTAs as potential tools for generating evidence to validate dismissed menstrual symptoms. Claudia demonstrated how algorithms could flag concerns: "Concerning symptoms presented over a period of time, please consult a doctor." However, participants suggested AI-generated evidence could become compulsory. Hannah captured how this could further obstruct medical belief in lived experience: "It feels like no one believes me unless it's logged" while Maya questioned the necessity: "Could you just trust my assessment without needing technology to validate it?" These reflections demonstrate how technological validation could become prerequisite for clinical belief. While participants were speculatively reflecting on AI-integrated MTAs, their concerns mirror clinical patterns, where people with a menstrual concern must provide a history of symptoms to a clinician. However, as accounts of symptoms and lived experience are often dismissed [5], AI-generated evidence could become a barrier to clinical belief.

3.2 Quantifying Menstrual Experiences

Participants expressed how AI-integrated MTAs may reduce embodied experiences to quantified data. Elena described how missed logs may lead to misrepresentation of symptoms: "I think tracking... may not capture the whole picture as it can come in waves/be tiring to remember to track." In another reflection, Elena reiterated that algorithms are "unable to capture the full picture." This suggests algorithms trained on standardized datasets may not recognize the heterogeneity of menstrual experiences [18]. Other participants had difficulties in responding to the prompt in Postcard One. Talia struggled to describe menstrual symptoms without using the word "pain", writing and crossing out the word: "Extreme ~~pain~~ discomfort in uterus/ovaries with excess bleeding." This shows how the existing descriptors might not capture the complexity and variability of menstrual symptoms. These reflections suggest that AI-integrated MTAs risk reducing qualitative menstrual experiences to quantified data, where symptoms are misunderstood and invalidated, excluding those whose experiences are not represented in datasets.

3.3 Reinforcing Reproductive labor

Participants described how AI-integrated MTAs shift tracking responsibility to people with menstrual conditions and concerns. Claudia framed tracking as compulsory administrative work: "I feel like unless I do the admin job of logging my periods and symptoms I will be dismissed." Ana explained how tracking could compound existing burdens: "It's just a pain – a different sort of pain, I guess – to have to constantly be thinking about it and remembering it and knowing that if I forget to track it, I'll be somehow less legitimate. It's another burden when I already have enough to carry." The metaphor of "a different sort of pain" reveals how tracking itself might become another symptom to manage for people with a menstrual condition or concern. Ana's reflection also captures how non-compliance with tracking could reduce credibility of experiences. Talia rejected the expectation that patients must educate clinicians: "It is not our

job to educate you." These reflections extend feminist critiques of self-tracking [15] revealing how AI could intensify reproductive labor rather than reduce barriers to care.

3.4 A Sociotechnical Hierarchy of Trust

Participants reflected on their trust in AI-integrated MTAs and clinicians with their lived experiences. Elena positioned AI as more reliable than male doctors while remaining subordinate to clinicians with a uterus: *"I think it's mixed if I trust it more than a doctor, generally I'd likely trust doctors more but male doctors can be ignorant when it comes to people w/ uterus."* Talia extended this skepticism, questioning whether either algorithms or clinicians possess genuine understanding: *"It may keep an accurate timeline of symptoms, but does it really understand? Then again, I don't know if the real doctors themselves really understand."* These reflections highlight a hierarchy of trust across menstrual healthcare, leaving patients navigating between inadequate options rather than accessing care that prioritizes their needs.

4 Discussion

Across all four themes, participants demonstrated critical reflection on AI-integrated MTAs in medical dismissal. Participants described AI-generated evidence becoming 'compulsory', with data superseding lived experience. This linked to concerns around quantified data reducing qualitative experiences. Participants highlighted intensified reproductive labor with menstrual tracking, adding administrative work rather than reducing barriers. Finally, they revealed complex hierarchies of trust across clinicians and algorithms. Overall, the participants reflected on how AI-integrated MTAs may perpetuate medical dismissal and gaslighting.

4.1 Reflections and Future Directions for HCI

We used handwritten postcards to engage participants in slow reflection on AI-integrated MTAs. Participants placed their written reflections into sealed envelopes which created space to articulate their perspectives on the ways in which AI-enhanced MTAs might reproduce, exacerbate, or challenge existing epistemic injustice and bias. The numbered envelopes allowed participants to return to their thoughts at different times, rather than a single interview or questionnaire which may have captured evolving thoughts about AI-powered MTAs. In addition, the private, self-paced nature of the postcard format provided a space to explore their speculations and imaginaries around AI-integrated MTAs without having to disclose any diagnoses, concerns, or experiences.

However, we acknowledge that the framing of some prompts may have encouraged critical reflection on potential harms, which could have primed participants to focus on concerns rather than benefits. This critical stance was grounded in feminist HCI [27] and literature documenting the dismissal of menstrual conditions [33]. While our findings centre critical perspectives on these technologies, AI-integrated MTAs are likely to continue to proliferate. As such, future work might employ more neutrally framed probes to explore people's views of AI integration in MTAs and menstrual healthcare more fully.

We also note that this method captured participants' anticipated concerns about AI-integrated MTAs rather than lived experiences

of use in clinical encounters. As such, we offer the insights not as a basis for foregrounding design tensions and actionable design questions for practitioners working in this space. In addition, future research should conduct user studies to investigate whether AI-integrated MTAs shift symptom-tracking and validating responsibility to people with menstrual conditions or concerns navigating medical dismissal.

While MTAs are positioned as empowering tools for health knowledge [11], our findings suggest that AI-integrated MTAs could intensify reproductive labor, and make technological evidence compulsory. This reflects '*Yentl syndrome*', whereby women must prove themselves to be unwell for their illness to be taken seriously [9]. The findings also revealed a sociotechnical hierarchy of trust in AI-integrated MTAs and clinicians in menstrual healthcare. This could be shaped by medical misogyny which has significantly undermined trust in the healthcare system [17]. Participants might be comparing AI systems favorably to human doctors because clinicians could dismiss their symptoms and pain rather than address them [33]. Despite the potential for harm, there is limited guidance for developers in identifying and mitigating bias. While frameworks exist to guide representative datasets in AI for AFAB people's health technologies [10], these address algorithmic bias rather than examining how technologies might reproduce or challenge medical misogyny, bias, and reproductive labor. As such, we suggest future work should develop a framework to evaluate how AI-integrated MTAs might perpetuate or challenge biases in menstrual healthcare access during the design process.

4.2 Conclusion

This qualitative study demonstrates that AI-integrated MTAs may amplify epistemic injustices in menstrual healthcare, including intensifying reproductive labor, reducing lived experiences to data, making evidence generation compulsory, and creating hierarchies of trust shaped by medical misogyny. Drawing on these reflections, future HCI research should conduct user studies exploring whether AI-integrated MTAs shift reproductive labor to those with menstrual concerns, and develop a framework for designers to evaluate how these technologies might perpetuate or challenge medical biases. Addressing these recommendations could lead to more inclusive and supportive tools for those navigating healthcare systems with menstrual conditions or concerns.

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